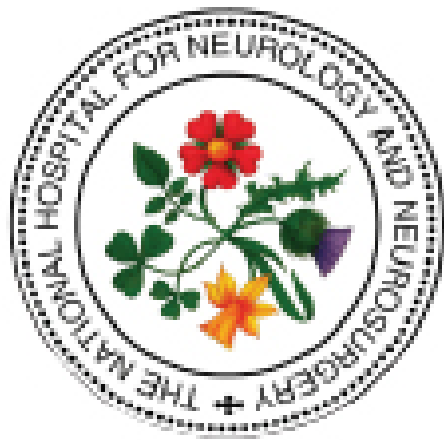




The National Hospital for Neurology and Neurosurgery Neuroscience Critical Care Unit (NCCU)

Death by Circulatory Death (DCD) - APPROACHING FAMILIES of POTENTIAL DCD DONORS FOLLOWING WITHDRAWAL of TREATMENT DECISION (*Collaborative Approach*)

Once decision made by **Neurocritical Care (NCC) MDT** to withdraw life-sustaining treatment in a patient with life-threatening or life-limiting condition which will, or is expected to, result in circulatory death, refer to the Embedded Specialist Nurse – Organ Donation (SNOD) or on call SNOD – Pager **03000 20 30 40**

NCC Nurse, NCC Consultant +/- SNOD discuss MDT decision / plan to withdraw treatment with family

DONATION DISCUSSION IS NOT INITIATED AT THIS TIME

- When family agree with the MDT opinion that continuing treatment is not in patient's best interests, NCC Consultant, NCC Nurse and SNOD discuss donation with family
- SNOD gives family information on patient's wishes, where known
- Family supported throughout decision-making process
- Family given relevant information and any questions addressed at this time
- Handprints / hair locks can be offered to family at this time (see *SWAN box*) and death certificate completed if **NOT** coroner's case

If family decide donation is not right option for them, they are thanked for considering donation and treatment is discontinued

Refer to palliative extubation checklist



Family are supported by NCC nurse +/- SNOD

If patient dies within short time, 'After Death Care'-performed

If death not imminent, arrangements may be made to transfer patient to a side room on a ward

If family consent to donation the SNOD works with the MDT to optimise the patient for donation as per 'Management of Patient For Organ Donation'

- It is acceptable to maintain the existing level of support, and consider introduction / escalation of treatment if needed (*Donation Actions Framework 2022*)
- NCC medical staff contact Duty Medical Examiner - *all NCC deaths discussed with UCLH Medical Examiner – via email: uclh.medicalexaminer@nhs.net / or Tel: 020 344(75129) and 020 344(75130) Monday – Friday During extended hours please call 020 3086 7056, 9am-12noon*
- The SNOD:
 - informs family of process of organ donation
 - completes Consent and Patient Assessment with family
 - contacts Coroner and GP (*where able*)
 - undertakes physical examination and assessment of patient
 - takes blood samples for virology and tissue typing
- The patient is registered with Hub Operations, where matching process commenced and organs offered - SNOD will liaise with theatres, anaesthetics and organise retrieval teams to attend hospital
- Once retrieval teams are ready, withdrawal takes place either in NCC (*1st floor unit if possible*) or in the anaesthetic room and this decision should be taken by the clinical team on duty – *NCC nurse remains responsible for patient (wherever withdrawal takes place) until patient certified dead*
- If cardiac / lung retrieval planned, extubation **must** take place in the anaesthetic room
- Family can be present at this time, either in NCC or in the anaesthetic room
- Following loss of cardiac output, patient monitored for further **5 minutes**
- Certification of death in accordance with usual cardiorespiratory criteria either by the Anaesthetic SpR attending in theatre or by a member of NCC medical team (ST3 or above) who knows the patient
- If, at this time, family do not wish to leave, retrieval teams stand down and donation process stopped
- If retrieval process stopped, patient will return to NCC for 'After Death Care'
- If donation proceeds where lung retrieval anticipated, patient will be reintubated post confirmation of circulatory death, prior to retrieval process – *but lung insufflation will only be performed 10 minutes after loss of output*
- If donation proceeds, once completed, SNOD performs 'After Death Care' in theatre, with theatre staff and organises transfer to mortuary
- If patient does not die within stipulated time, they return to NCC for continuing 'end of life' care

REFERRAL to SNOD

The medical decision made by NCC Consultant and referring teams that continuing life sustaining treatment no longer in patient's best interests, but treatment will not yet have been withdrawn

Organ donation should **NOT** be discussed in same conversation as withdrawal of life-sustaining treatment unless those close to patient initiate these discussions

Vital that ventilation continues, and that patient is **NOT** extubated at point the referral is made

Ultimate responsibility for referral of patient to SNOD lies with NCC Consultant - *however, any staff member involved in the care of the patient can make the referral*

WITHDRAWAL of TREATMENT

The continuation of good end of life care, including measures to maintain the comfort and dignity of the patient should **never** be compromised for the purposes of facilitating organ donation

Once consent has been obtained it is acceptable to maintain the existing level of support, and introduction of additional treatment may also be considered

Neither the SNOD nor any member of the surgical retrieval team will be involved with the withdrawal process

CERTIFYING DEATH

Following onset of **monitored loss of output**, patient should continue to be monitored for 5 minutes. If, during this time, there is any return of **cardiac output** a further 5 minutes of observation should recommence.

At the conclusion of 5 minutes of monitored asystole death can be certified in accordance with usual cardiorespiratory criteria by an appropriately trained doctor involved in the patient's care.

In circumstances where lung donation is anticipated, the anaesthetist will be required to reintubate the patient in theatre after confirmation of death and prior to the retrieval process.

